ALVERA

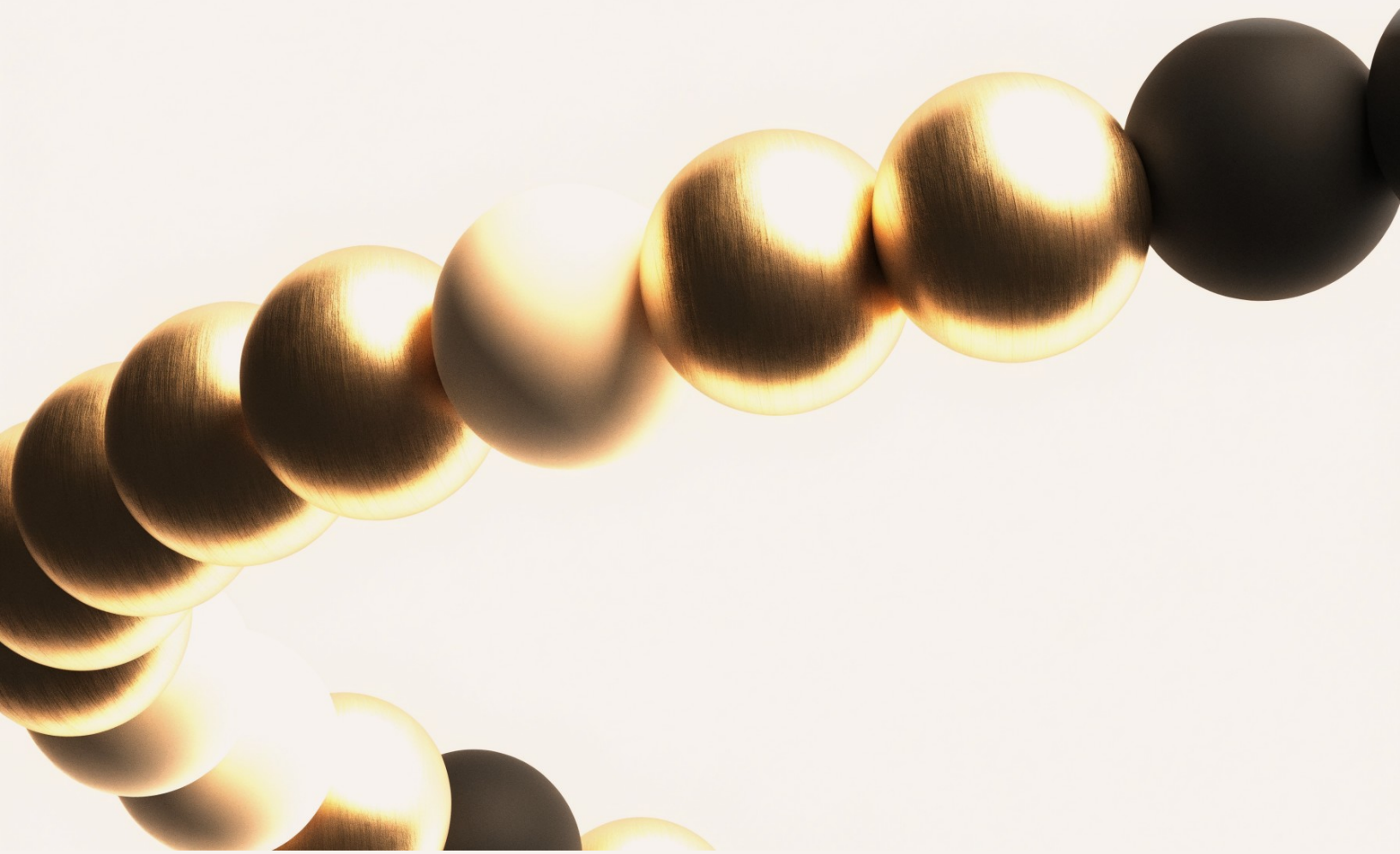
CLINICAL

INTERNAL REFERENCE · NOT FOR DISTRIBUTION

# Peptides, *made simple.*

What each product is, what people use it for, and the published dosing protocols: how much, how often, and how long.

**Protocols compiled from published vendor/clinic sources; not validated human doses.**



Internal edition · October 8, 2026 · Internal reference. Not medical advice.

# Peptide protocols at a glance

**Units** = dose ÷ (mg per mL) × 100 · 1 mg = 1,000 mcg · 100 units = 1 mL

| PRODUCT | WHAT PEOPLE USE IT FOR | TYPICAL DOSE | HOW OFTEN | CYCLE / OFF | EXAMPLE MIX (BAC WATER) | UNITS ON U-100 | SOURCE |
|---|---|---|---|---|---|---|---|
| **Weight & metabolism** | | | | | | | |
| **Semaglutide** | Weight loss, appetite control, blood-sugar control. | **0.25 mg → step up to 1–2.4 mg** | Once weekly | +1 step every 4 wks; ongoing | 5 mg + 2 mL = 2.5 mg/mL | 0.25 mg = 10 u<br>1 mg = 40 u | FDA label (Ozempic/ Wegovy) |
| **Tirzepatide** | Weight loss, appetite control, blood-sugar control. | **2.5 mg → step up to 5–15 mg** | Once weekly | +2.5 mg every 4+ wks; ongoing | 10 mg + 1 mL = 10 mg/mL | 2.5 mg = 25 u<br>5 mg = 50 u | FDA label (Mounjaro/ Zepbound) |
| **Retatrutide** | Weight loss, appetite control, blood-sugar control. | **2 mg start → 4, 8 or 12 mg (trial)** | Once weekly | 48 wks (trial length) | 30 mg + 3 mL = 10 mg/mL | 2 mg = 20 u<br>4 mg = 40 u | Phase 2 trial, NEJM 2023 |
| **Lipo-C / B12** | Energy, fat-metabolism support alongside a weight-loss plan. | **1 mL (range 0.5–2 mL)** | 1–2× weekly | None stated | Pre-mixed liquid | 1 mL = 100 u | MediVera Rx |
| **5-Amino-1MQ** | Fat loss, metabolism and energy. | **SubQ 2.5–5 mg · oral 50–150 mg** | Daily (morning) | 8–12 wks on / 4–6 off | 10 mg + 1 mL = 10 mg/mL | 2.5 mg = 25 u<br>5 mg = 50 u | PeptideDosingProtocols · YooDirect Health |
| **Growth-hormone axis** | | | | | | | |
| **Tesamorelin** | Belly (visceral) fat loss, body composition, energy. | **1 mg (clinic) · 1.4 mg (label)** | Bedtime, 5 on / 2 off | 3 months / 2 off | 10 mg + 2.5 mL = 4 mg/mL | 1 mg = 25 u<br>1.4 mg = 35 u | FDA label (Egrifta) · Perfect B |
| **Tesamorelin + Ipamorelin** | Belly fat loss, lean muscle, deeper sleep, recovery. | **1 mg tesa (mo 1) → 1.5 mg** | Bedtime, 5 on / 2 off | 3 months / 2 off | 20 mg (10+10) + 2 mL = 5 mg/mL each | 1 mg = 20 u<br>1.5 mg = 30 u | Perfect B |
| **CJC-1295 / Ipamorelin** | Sleep, recovery, lean muscle, fat loss. | **0.3 mg → 1.0 mg blend** | Bedtime, 5 on / 2 off | 3 months / 2 off | 10 mg + 2 mL = 5 mg/mL | 0.3 mg = 6 u<br>1 mg = 20 u | Perfect B |
| **Ipamorelin** | Deeper sleep, recovery, lean muscle. | **100 mcg → 100–300 mcg** | Bedtime, 1–3× daily, 5 d/wk | 8–12 wks / 4+ off | 10 mg + 4 mL = 2.5 mg/mL | 100 mcg = 4 u<br>300 mcg = 12 u | Beverly Hills Rejuvenation Ctr. |
| **Sermorelin** | Better sleep, energy, muscle tone, fat loss, healthy aging. | **200–300 mcg (up to 500)** | Bedtime, 5 on / 2 off | 3–6 months | 15 mg + 7.5 mL = 2 mg/mL | 200 mcg = 10 u<br>300 mcg = 15 u | Eden |

Every dose, schedule and cycle comes from the source named in that row (fetched Oct 8, 2026). The mix and units columns are just the math for that example mix; change the water and the units change. “NTN” means the off-period came from the r/NTNPerformance Peptide Master Cheat Sheet because the main source didn't give one. Blends are dosed by total mg unless noted. General dosing reference: Path to Peptides calculator.

# Peptide protocols at a glance

**Units** = dose ÷ (mg per mL) × 100 · 1 mg = 1,000 mcg · 100 units = 1 mL

| PRODUCT | WHAT PEOPLE USE IT FOR | TYPICAL DOSE | HOW OFTEN | CYCLE / OFF | EXAMPLE MIX (BAC WATER) | UNITS ON U-100 | SOURCE |
|---|---|---|---|---|---|---|---|
| **Recovery & repair** | | | | | | | |
| **BPC-157** | Injury recovery, tendon and ligament support, joint pain, gut health. | **250–500 mcg** | 1–2× daily | 6–12 wks / 2–4 off (NTN) | 10 mg + 2 mL = 5 mg/mL | 250 mcg = 5 u<br>500 mcg = 10 u | PS Peptides · NTN |
| **TB-500** | Injury recovery, muscle and soft-tissue repair, flexibility. | **2–2.5 mg** | 2× weekly (load) → weekly | 6–12 wks / 2–4 off (NTN) | 10 mg + 2 mL = 5 mg/mL | 2 mg = 40 u<br>2.5 mg = 50 u | PS Peptides · NTN |
| **"Wolverine" blend** | Injury recovery, joint and tendon support, faster bounce-back from training. | **250–500 mcg of each part** | Daily, or 5 on / 2 off | 3 months / 2–4 off (NTN) | 10 mg (5+5) + 2 mL = 2.5 mg/mL each | 250 mcg = 10 u<br>500 mcg = 20 u | PS Peptides · Perfect B stack guide · NTN |
| **Skin & appearance** | | | | | | | |
| **GHK-Cu** | Skin firmness and texture, collagen, fine lines, hair thickness. | **1 mg (days 1–15) → 2 mg (16–30)** | Daily, bedtime | 30 days on / 30 off | 50 mg + 2 mL = 25 mg/mL | 1 mg = 4 u<br>2 mg = 8 u | Perfect B |
| **GLOW 70 mg** | Skin quality and glow, collagen, plus recovery from training and nagging injuries. | **1.17 mg start → 2.33 mg blend** | Daily (or weekdays) | 4–12 wks / 2–4 off | 70 mg + 3 mL = 23.3 mg/mL | 1.17 mg = 5 u<br>2.33 mg = 10 u | SeekPeptides · Drip Hydration · Pure Hydration |
| **KLOW 80 mg** | Injury recovery and inflammation, gut comfort, plus skin and collagen. | **2 mg → 3 mg → 4 mg blend** | Mon–Fri, or daily | 6–8 wks / 2–6 off | 80 mg + 4 mL = 20 mg/mL | 2 mg = 10 u<br>4 mg = 20 u | SeekPeptides · PeRx · ElliEMD |
| **Glutathione** | Skin brightening, "detox" and liver support, recovery, immune support. | **100–300 mg (up to 400–800)** | Daily | No set cycle | 600 mg + 3 mL = 200 mg/mL | 100 mg = 50 u<br>200 mg = 100 u | Cody Drug |
| **Energy & longevity** | | | | | | | |
| **NAD+** | Energy, mental clarity, healthy aging, recovery. | **50 mg start · 100–200 mg** | Daily 7–14 d, then 2–3× wk | Ongoing; none stated | 500 mg + 5 mL = 100 mg/mL | 50 mg = 50 u<br>100 mg = 100 u | Atlas Men's Health |
| **MOTS-c** | Energy, metabolism and blood sugar, endurance, fat loss. | **5 mg** | Every 5 days × 4 shots | 20 days on / 4 months off | 10 mg + 0.5 mL = 20 mg/mL | 5 mg = 25 u | Perfect B |
| **Focus & mood** | | | | | | | |
| **Selank** | Calm focus, mental clarity, stress and anxiety. | **250 mcg → 500 mcg SubQ** | Daily | 4 wks on / 4 off | 10 mg + 4 mL = 2.5 mg/mL | 250 mcg = 10 u<br>500 mcg = 20 u | PeptideDosingProtocols |

Every dose, schedule and cycle comes from the source named in that row (fetched Oct 8, 2026). The mix and units columns are just the math for that example mix; change the water and the units change. "NTN" means the off-period came from the r/NTNPerformance Peptide Master Cheat Sheet because the main source didn't give one. Blends are dosed by total mg unless noted. General dosing reference: Path to Peptides calculator.

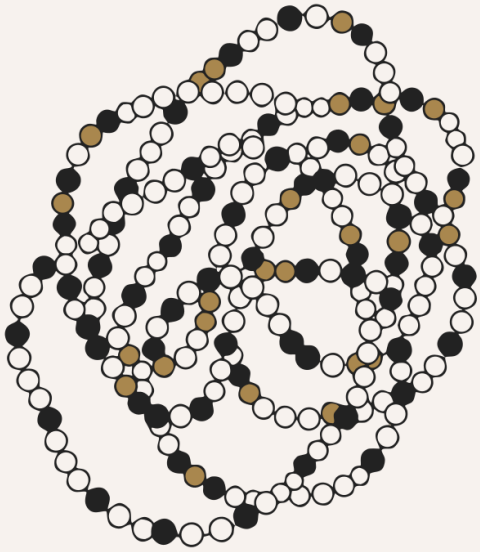
# What is a peptide?

Think of **amino acids as beads**. String a few beads together and you get a **peptide**, like a short bracelet. String hundreds together and fold them up, and you get a **protein**, like a long necklace balled up in a drawer.



### Peptide

a short chain of amino acids



### Protein

a long chain, folded into a shape

**Your body makes them**

Many peptides work as **signals**. They carry short messages, like “I'm full” or “release growth hormone.” GLP-1 is one: a gut hormone that helps control appetite.

**Labs can copy them**

Scientists can build a peptide bead by bead. Some copies became prescription drugs, like semaglutide and tirzepatide.

**Not all are peptides**

Some products sold next to peptides aren't peptides at all. **NAD+** is a coenzyme. **5-Amino-1MQ** is a small chemical.

SAY IT IN 10 SECONDS

“A peptide is a short chain of amino acids, the same building blocks as protein. Your body uses them as signals, and labs can make copies.”

HOW TO READ THE PROTOCOL CARDS

Each card has **what it is**, **what people use it for**, the **published protocol** (route, dose, how often, cycle and off-time), and a **mix & units** example. The source is linked at the bottom of every card. Where sources disagree, the card shows each one by name.

PROTOCOLS BY GOAL

# Weight & metabolism

Appetite, blood sugar and fat storage. The GLP-1 family is dosed once a week and stepped up slowly.

## Semaglutide

GLP-1 signal · weekly shot

**A long-acting copy of GLP-1, the gut hormone that controls appetite and blood sugar.**

**WHAT PEOPLE USE IT FOR** Weight loss, appetite control, blood-sugar control.

**FDA LABEL DOSING**

| | |
|---|---|
| ROUTE | Under the skin (SubQ), once a week. |
| DOSE | **Ozempic:** 0.25 mg weekly for 4 wks, then 0.5 mg; step to 1 mg, then 2 mg max, at least 4 wks apart. **Wegovy:** 0.25 → 0.5 → 1 → 1.7 → 2.4 mg, stepping up every 4 wks; maintain at 2.4 mg (1.7 mg if needed). |
| CYCLE | Ongoing at the maintenance dose. No off-period on the label. |

**Mix & units:** 5 mg vial + 2 mL = 2.5 mg/mL → 0.25 mg = **10 units**, 1 mg = **40 units**.

Source: FDA label (Ozempic/Wegovy)

## Tirzepatide

GIP + GLP-1 signal · weekly shot

**One molecule that switches on two gut-hormone signals (GIP and GLP-1).**

**WHAT PEOPLE USE IT FOR** Weight loss, appetite control, blood-sugar control.

**FDA LABEL DOSING**

| | |
|---|---|
| ROUTE | SubQ, once a week. |
| DOSE | 2.5 mg weekly for 4 wks, then 5 mg. Step up 2.5 mg at a time, at least 4 wks apart. Maintenance 5, 10 or 15 mg (15 mg max). |
| CYCLE | Ongoing at the maintenance dose. No off-period on the label. |

**Mix & units:** 10 mg + 1 mL (or 30 mg + 3 mL) = 10 mg/mL → 2.5 mg = **25 units**, 5 mg = **50 units**.

Source: FDA label (Mounjaro/Zepbound)

**HOW THE GLP-1 FAMILY WORKS**

1. **Raises insulin after meals**
2. **Slows how fast the stomach empties**
3. **Turns down appetite**

**Semaglutide** copies GLP-1. **Tirzepatide** adds a second signal (GIP). **Retatrutide** adds a third (glucagon).

## Retatrutide

GIP + GLP-1 + glucagon · weekly shot

**Eli Lilly's weekly shot that hits three hormone signals. Doses below are from its Phase 2 trial.**

**WHAT PEOPLE USE IT FOR** Weight loss, appetite control, blood-sugar control.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| ROUTE | SubQ, once a week. |
| DOSE | Trial arms: 1, 4, 8 or 12 mg once weekly. Some groups started at 2 mg and stepped up to their target. |
| CYCLE | 48 weeks in the trial. |

**Mix & units:** 30 mg + 3 mL = 10 mg/mL → 2 mg = **20 units**, 4 mg = **40 units**, 8 mg = **80 units**.

Source: Phase 2 trial, NEJM 2023

## Lipo-C / B12

B12 + lipotropic nutrients · liquid

**Vitamin B12 with "fat-mobilizing" nutrients (methionine, choline, carnitine and similar). Arrives as a liquid, no mixing.**

**WHAT PEOPLE USE IT FOR** Energy, fat-metabolism support alongside a weight-loss plan.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| ROUTE | Into the muscle (IM) or SubQ. |
| DOSE | Start at 1 mL, 1–2 times a week. Range 0.5–2 mL; adjust by 0.5 mL. |
| CYCLE | No cycle or off-period stated. |

**Mix & units:** Liquid: 1 mL = **100 units**, 0.5 mL = **50 units**.

Source: MediVera Rx

## 5-Amino-1MQ

Small molecule (not a peptide)

**A small chemical that blocks NNMT, an enzyme tied to fat storage. Used as a capsule or a shot.**

**WHAT PEOPLE USE IT FOR** Fat loss, metabolism and energy.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| ROUTE | By mouth (capsule) or SubQ. The two doses are not interchangeable. |
| DOSE | **Oral:** 50–150 mg a day, in the morning. **SubQ:** 2.5–5 mg a day. |
| CYCLE | 8–12 weeks on (2–3 months), then 4–6 weeks off. |

**Mix & units:** 10 mg vial + 1 mL = 10 mg/mL → 2.5 mg = **25 units**, 5 mg = **50 units**.

Source: PeptideDosingProtocols · YooDirect Health

**QUICK TAKE**

The GLP-1 shots are weekly and step up every 4 weeks. Most others in this guide are daily or 5 days on, 2 off.

PROTOCOLS BY GOAL

# Recovery & repair

The injury and recovery peptides. BPC-157 is dosed daily; TB-500 is loaded twice a week, then tapered.

## BPC-157

15-amino-acid peptide

**A lab-made peptide based on a protein found in stomach juice.**

**WHAT PEOPLE USE IT FOR** Injury recovery, tendon and ligament support, joint pain, gut health.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| ROUTE | SubQ, often near the injured area. |
| DOSE | 250–500 mcg per dose, once or twice a day (e.g. 250 mcg twice daily). Short half-life (about 4–6 hrs), so it is dosed daily. |
| CYCLE | 6–12 weeks (repair protocols). Off-period not stated; NTN cheat sheet lists 2–4 weeks. |

**Mix & units:** 10 mg + 2 mL = 5 mg/mL → 250 mcg = **5 units**, 500 mcg = **10 units**.

Source: PS Peptides · NTN cheat sheet

## TB-500

Based on thymosin beta-4

**Based on a natural protein that helps cells move into a wound to repair it.**

**WHAT PEOPLE USE IT FOR** Injury recovery, muscle and soft-tissue repair, flexibility.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| ROUTE | SubQ. |
| DOSE | **Loading:** 2–2.5 mg twice a week for about 4–6 weeks. **Maintenance:** 2–2.5 mg once a week or every other week. |
| CYCLE | 6–12 weeks total. Off-period not stated; NTN cheat sheet lists 2–4 weeks. |

**Mix & units:** 10 mg + 2 mL = 5 mg/mL → 2 mg = **40 units**, 2.5 mg = **50 units**.

Source: PS Peptides · NTN cheat sheet

## “Wolverine” blend

BPC-157 + TB-500, 1:1 in one vial

**BPC-157 and TB-500 pre-mixed. The ratio is fixed, so both parts move together.**

**WHAT PEOPLE USE IT FOR** Injury recovery, joint and tendon support, faster bounce-back from training.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| **ROUTE** | SubQ, before bed. |
| **DOSE** | Dose by the BPC-157 part: 250–500 mcg, once or twice a day (PS Peptides). A 1:1 vial gives the same amount of TB-500. |
| **CYCLE** | Perfect B runs the blend 5 days on / 2 off, for 3 months. Off-period not stated; NTN lists 2–4 weeks for each part. |

**Mix & units:** 10 mg (5+5) + 2 mL = 2.5 mg/mL of each → 250 mcg each = **10 units**, 500 mcg each = **20 units**.

Source: PS Peptides · Perfect B stack guide · NTN cheat sheet

**QUICK TAKE**

BPC-157 is daily in mcg. TB-500 is twice a week in mg. Don't mix up the two scales when drawing.

PROTOCOLS BY GOAL

# Skin & appearance

Built around GHK-Cu, the copper peptide. GLOW and KLOW add the recovery peptides on top. Blends are dosed by the total mg.

## GHK-Cu

Copper peptide (3 amino acids)

**A tiny copper-carrying peptide found naturally in blood. Common in skin creams.**

**WHAT PEOPLE USE IT FOR** Skin firmness and texture, collagen, fine lines, hair thickness.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| **ROUTE** | SubQ, in the belly, before bed (2+ hrs after eating). |
| **DOSE** | 1 mg a day for days 1–15, then 2 mg a day for days 16–30. |
| **CYCLE** | 30 days on, 30 days off, then repeat. Dosed every day, not 5/2. |

**Mix & units:** 50 mg + 2 mL = 25 mg/mL → 1 mg = **4 units**, 2 mg = **8 units**.

Source: Perfect B

## GLOW 70 mg

GHK-Cu 50 + BPC-157 10 + TB-500 10 mg

**Three peptides in one vial: the skin peptide plus the two repair peptides.**

**WHAT PEOPLE USE IT FOR** Skin quality and glow, collagen, plus recovery from training and nagging injuries.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| **ROUTE** | SubQ, once a day (Pure Hydration: weekday mornings, weekends off). |
| **DOSE** | **Start:** 1.17 mg blend a day. **Standard:** 2.33 mg a day (≈1.7 mg GHK-Cu + 330 mcg each BPC-157 and TB-500). |
| **CYCLE** | 4 weeks on, 2–4 weeks off (SeekPeptides). Others run 4–8 wks (Drip) or 6–12 wks (Pure), with a 2–4 wk break. |

**Mix & units:** 70 mg + 3 mL = 23.3 mg/mL → 1.17 mg = **5 units**, 2.33 mg = **10 units**.

Source: SeekPeptides · Drip Hydration · Pure Hydration

## KLOW 80 mg

GHK-Cu 50 + BPC-157 10 + TB-500 10 + KPV 10 mg

**GLOW plus KPV, a tiny peptide used for calming inflammation. 80 mg total.**

**WHAT PEOPLE USE IT FOR** Injury recovery and inflammation, gut comfort, plus skin and collagen.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| ROUTE | SubQ, 5 days a week (Mon–Fri) or daily. |
| DOSE | Step up (SeekPeptides): 1.5–2 mg blend a day wks 1–2, 3 mg wks 3–4, 4–4.5 mg wks 5–8. PeRx's fixed dose is about 3.8 mg (its "20 units" is from its own premixed vial). |
| CYCLE | 6–8 weeks on. Off: 2 wks (ElliEMD), 3–4 wks after 8 on (SeekPeptides), or 6 wks (PeRx). |

**Mix & units:** 80 mg + 4 mL = 20 mg/mL → 2 mg = **10 units**, 3 mg = **15 units**, 4 mg = **20 units**.

Source: SeekPeptides · PeRx · ElliEMD

## Glutathione

Antioxidant (3 amino acids)

**The body's main built-in antioxidant, made by the liver.**

**WHAT PEOPLE USE IT FOR** Skin brightening, "detox" and liver support, recovery, immune support.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| ROUTE | Injection, once a day, morning or evening. |
| DOSE | **Low:** 100–300 mg a day. **Moderate:** 400–800 mg. **High:** 900–1,500 mg (supervised plans). |
| CYCLE | No set cycle. Some run a higher-dose phase, then maintenance; others stay on steady support. |

**Mix & units:** 600 mg + 3 mL = 200 mg/mL → 100 mg = **50 units**, 200 mg = **100 units** (1 mL).

Source: Cody Drug

PROTOCOLS BY GOAL

# Growth-hormone axis

These nudge the body to release more of its own growth hormone. Most run at bedtime on an empty stomach, 5 days on and 2 off.

## Tesamorelin

GHRH copy

**A stable copy of the brain hormone that tells the body to release growth hormone.**

**WHAT PEOPLE USE IT FOR** Belly (visceral) fat loss, body composition, energy.

**FDA LABEL DOSING + CLINIC PROTOCOL**

| | |
|---|---|
| LABEL | **Egrifta SV:** 1.4 mg SubQ once daily. **Egrifta WR:** 1.28 mg once daily (not interchangeable). |
| CLINIC | **Perfect B:** 1 mg before bed (2+ hrs after eating), 5 days on / 2 off. |
| CYCLE | Perfect B: monthly blocks of 20 shots, up to 3 months, then a 2-month break. |

**Mix & units:** 10 mg + 2.5 mL = 4 mg/mL → 1 mg = **25 units**, 1.4 mg = **35 units**.

Source: FDA label (Egrifta) · Perfect B

## Tesamorelin + Ipamorelin

Our vial: 10 mg + 10 mg (1:1)

**Tesamorelin plus ipamorelin in one vial for a stronger growth-hormone pulse.**

**WHAT PEOPLE USE IT FOR** Belly fat loss, lean muscle, deeper sleep, recovery.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| ROUTE | SubQ, before bed, 5 days on / 2 off. |
| DOSE | Perfect B: 1 mg tesamorelin in month 1, then 1.5 mg in months 2–3. Their vial is 2:1, so they get half as much ipamorelin. In our 1:1 vial, each mg of tesamorelin carries 1 mg of ipamorelin. |
| CYCLE | 3 months, then a 2-month break. |

**Mix & units:** 20 mg (10+10) + 2 mL = 5 mg/mL of each → 1 mg = **20 units**, 1.5 mg = **30 units**.

Source: Perfect B

## CJC-1295 / Ipamorelin

10 mg blend (5 + 5)

**CJC-1295 stretches out the growth-hormone signal; ipamorelin triggers the pulse.**

**WHAT PEOPLE USE IT FOR** Sleep, recovery, lean muscle, fat loss.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| **ROUTE** | SubQ, before bed (2+ hrs after eating), 5 days on / 2 off. |
| **DOSE** | 0.3 mg blend a day in week 1, up 0.1 mg each week, to 1.0 mg by month 2. Hold there in month 3. |
| **CYCLE** | Monthly blocks of 20 shots, up to 3 months, then a 2-month break. |

**Mix & units:** 10 mg + 2 mL = 5 mg/mL → 0.3 mg = **6 units**, 1 mg = **20 units**.

Source: Perfect B

## Ipamorelin

Ghrelin-mimic

**Copies the hunger hormone ghrelin to trigger a clean growth-hormone pulse.**

**WHAT PEOPLE USE IT FOR** Deeper sleep, recovery, lean muscle.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| **ROUTE** | SubQ, at bedtime on an empty stomach (2–3 hrs after carbs). |
| **DOSE** | Start 100 mcg once nightly. Working range 100–300 mcg, 1–3 times a day. |
| **CYCLE** | 5 days a week (up to 7), for 8–12 weeks, then about 4+ weeks off. |

**Mix & units:** 10 mg + 4 mL = 2.5 mg/mL → 100 mcg = **4 units**, 300 mcg = **12 units**.

Source: Beverly Hills Rejuvenation Ctr.

## Sermorelin

Short GHRH (29 amino acids)

**A short version of the brain hormone that triggers growth-hormone release.**

**WHAT PEOPLE USE IT FOR** Better sleep, energy, muscle tone, fat loss, healthy aging.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| **ROUTE** | SubQ, at bedtime (9–11 PM), 1+ hr after a meal. |
| **DOSE** | Start 200–300 mcg; standard 300 mcg. Range 200–500 mcg. |
| **CYCLE** | 5 nights on / 2 off, for 3–6 months, with periodic breaks. |

**Mix & units:** 15 mg + 7.5 mL = 2 mg/mL (Eden's mix) → 200 mcg = **10 units**, 300 mcg = **15 units**.

Source: Eden

**QUICK TAKE**

Sermorelin: 200–300 mcg at bedtime, 5 nights on and 2 off, for 3–6 months.

PROTOCOLS BY GOAL

# Energy & longevity

Tied to how cells make energy. Very different schedules: NAD+ runs 2–3 times a week; MOTS-c is only 4 shots per cycle.

## NAD+

Coenzyme (not a peptide)

**A molecule every cell uses to make energy and repair DNA. Levels drop with age.**

**WHAT PEOPLE USE IT FOR** Energy, mental clarity, healthy aging, recovery.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| ROUTE | SubQ (IV is a separate clinic option). |
| DOSE | **Loading:** 100–200 mg a day for 7–14 days. **Maintenance:** start 50 mg, working dose 100–200 mg per shot. |
| CYCLE | Maintenance is 2–3 times a week. No off-period stated. |

**Mix & units:** 500 mg + 5 mL = 100 mg/mL → 50 mg = **50 units**, 100 mg = **100 units** (1 mL).

Source: Atlas Men's Health

## MOTS-c

Mitochondrial peptide

**A small peptide made by mitochondria, the cell's power plants.**

**WHAT PEOPLE USE IT FOR** Energy, metabolism and blood sugar, endurance, fat loss.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| ROUTE | SubQ in the belly, in the morning before the first meal. |
| DOSE | 5 mg per shot, every 5 days: days 1, 6, 11 and 16 (4 shots). Other guides range 2.5–10 mg per shot, 5–10 mg a week. |
| CYCLE | 20 days on, then 4 months off (about 2–3 cycles a year). |

**Mix & units:** 10 mg + 0.5 mL = 20 mg/mL (Perfect B's mix) → 5 mg = **25 units**.

Source: Perfect B

PROTOCOLS BY GOAL

# Focus & mood

Selank, sold in Russia as an anti-anxiety nasal spray.

**QUICK TAKE**

Selank SubQ: 250 mcg a day, stepping up to 500 mcg, 4 weeks on and 4 off.

## Selank

7-amino-acid peptide

**A peptide developed in Russia as an anti-anxiety nasal spray.**

**WHAT PEOPLE USE IT FOR** Calm focus, mental clarity, stress and anxiety.

**PUBLISHED PROTOCOL**

| | |
|---|---|
| ROUTE | SubQ or nasal spray. Pick one per cycle. |
| DOSE | **SubQ:** 250–300 mcg a day for wks 1–2, then 400–500 mcg wks 3–4. **Nasal:** 600–2,700 mcg a day, split into 2–3 sprays. |
| CYCLE | SubQ: 4 weeks on, 4 off (some use 5 on / 2 off). Nasal: 14–21 days on, 1–3 weeks off. |

**Mix & units:** 10 mg + 4 mL = 2.5 mg/mL → 250 mcg = **10 units**, 500 mcg = **20 units**.

Source: PeptideDosingProtocols

# The math of mixing

Most peptides ship as a dry powder. Adding bacteriostatic water turns it into a liquid. Two formulas cover everything.



**Strength (mg/mL) = mg in vial ÷ mL of water**

**Units (U-100) = dose ÷ strength × 100**

### Handy conversions

- 1 mg = 1,000 mcg
- On a U-100 insulin syringe, 100 units = 1 mL
- So 10 units = 0.1 mL
- Keep dose and strength in the same unit (both mcg or both mg)

### Worked example

1. BPC-157 **10 mg** + **2 mL** water.
2. 10 ÷ 2 = **5 mg/mL** = 5,000 mcg/mL.
3. Dose 250 mcg: 250 ÷ 5,000 × 100 = **5 units**.

## Units for the protocol doses in this guide

| Vial | + water | Strength | Dose | Math | Draw |
|---|---|---|---|---|---|
| **BPC-157 10 mg** | 2 mL | 5 mg/mL = 5,000 mcg/mL | 250 mcg | 250 ÷ 5,000 × 100 | **5 units** |
| **Ipamorelin 10 mg** | 4 mL | 2.5 mg/mL = 2,500 mcg/mL | 300 mcg | 300 ÷ 2,500 × 100 | **12 units** |
| **GHK-Cu 50 mg** | 2 mL | 25 mg/mL | 2 mg | 2 ÷ 25 × 100 | **8 units** |
| **Tesamorelin 10 mg** | 2.5 mL | 4 mg/mL | 1 mg | 1 ÷ 4 × 100 | **25 units** |
| **KLOW 80 mg** | 4 mL | 20 mg/mL (total) | 4 mg blend | 4 ÷ 20 × 100 | **20 units** |
| **MOTS-c 10 mg** | 0.5 mL | 20 mg/mL | 5 mg | 5 ÷ 20 × 100 | **25 units** |
| **Tirzepatide 10 mg** | 1 mL | 10 mg/mL | 2.5 mg | 2.5 ÷ 10 × 100 | **25 units** |

- **Blends:** the vial's mg is the total of all parts. KLOW + 4 mL = 20 mg/mL total: GHK-Cu 12.5, and BPC-157, TB-500, KPV 2.5 mg/mL each.
- **Big doses:** anything over 100 units (1 mL), like glutathione or NAD+, takes a stronger mix or two draws.
- **Storage:** powder cold, dry and dark; once mixed, keep at 2–8 °C. Swirl, don't shake. Bac water is 0.9% benzyl alcohol, for mixing only, “not for use in newborns.”

03 · HOW TO TALK ABOUT IT

# Say it the right way

People will ask what these do. These tips keep you accurate and believable.

### Do

- Say what people **use it for**, in plain words.
- Say “that was in mice” when it was.
- Point to the source protocol when asked how it's run.
- Point people to the free Path to Peptides dosing calculator.
- Show the COA when someone asks about quality.
- Point people to a doctor for health questions.

### Don't

- Say a product **“cures”** or “fixes” anything.
- Call a vial “Ozempic” or any brand name.
- Promise results, or say “natural, so no side effects.”
- Guess. “Let me check the source” is a fine answer.

| INSTEAD OF… | SAY… |
|---|---|
| ~~“BPC-157 heals injuries.”~~ | “People use BPC-157 for injury recovery. Most of the research is in animals.” |
| ~~“Retatrutide is the new Ozempic.”~~ | “Retatrutide is Lilly's next weekly shot. It's still in trials.” |

## Quick answers

**“Is this the same as Ozempic?”**

No. Ozempic and Wegovy are brand drugs sold by prescription. A peptide vial is not the brand drug.

**“How do I know it's real?”**

Every product has a COA (lab test report) showing the measured amount and purity.

**“How much do people take?”**

See the cheat sheet at the front. Each row links the clinic or vendor protocol it came from.

Protocols compiled Oct 8, 2026 from the vendor, clinic and label sources linked on each card. General dosing reference: pathtopeptides.com/DosingCalculator.html.